

A COMMUNITY FORUM HANDOUT

# BRINGING IT HOME

The Health Rosetta Dividend in a Single American County

DAVE CHASE

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*Mount Baker over Bellingham Bay, Whatcom County, Washington*

**Book Launch: July 29, 2026 | RosettaFest, Nashville**

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# The Problem We Already Pay For

*Two sides of a single broken system: dollars extracted from the community, and clinicians buried in the same machinery.*

Roughly \$3.9 billion flows through Whatcom County's healthcare system every year, based on a per capita spend of \$15,474 across 250,000 residents. Research from the Institute of Medicine, PwC, and three decades of employer implementations consistently finds that 30 to 50 percent of American healthcare spending produces no measurable value. Applied to Whatcom, that is up to **\$2 billion** in waste extracted from this community every year, flowing out to administrative overhead, pharmacy benefit managers, utilization management firms, and insurance companies headquartered far from the families paying the premiums.

## 15.6¢

Of every healthcare dollar that flows through Whatcom County, only 15.6 cents reaches a frontline clinician. The rest disappears into a labyrinth of extraction.

*Source: BLS Occupational Wage data, Bellingham MSA; CMS National Health Expenditure data*

## 2 : 1

For every hour a physician spends in direct patient care, they spend two hours on EHR and clerical work. Doctors did not choose this. Patients did not choose this.

*Source: AMA/Dartmouth study, Annals of Internal Medicine*

The dollars leaving every year are not theoretical. They are wages not paid, services not staffed, scholarships not funded, homes not weatherized, watersheds not restored, local health service providers not hired. And the clinicians who do reach patients are caught inside the same extraction machinery:

- **91 percent** of office-based physicians do clinical documentation outside normal office hours; **24 percent** do two to four hours per day of it; **8.6 percent** do more than four hours per day. (*CDC/NCHS, 2019*)
- **84.7 percent** of physicians say documentation done solely for billing purposes inflates their total documentation time. (*JAMA Internal Medicine*)
- Physicians and their staff spend **12 to 13 hours every week** on prior authorizations alone, and **94 percent** of physicians say prior authorization contributes directly to clinician burnout. (*AMA*)

These are not the symptoms of hard jobs. They are the symptoms of a payment model that rewards coding and volume over outcomes and trust. The community pays twice: once with the dollars that never come back, and again with the time and attention taken from the people who would otherwise be at the bedside.

## The Movement That Reclaims It

This pattern is reversible. It has been reversed, in communities across the country, for more than three decades.

Rosen Hotels in Orlando runs healthcare 55 percent below industry average and has cumulatively saved \$570 million, redirected into scholarships, neighborhood renewal, and one of the most transformed neighborhoods in the country. Phifer Incorporated in Alabama held per-employee healthcare costs flat for five consecutive years, funding scholarships for 100 employee dependents, summer enrichment for 200 children, free pharmacy for 1,700 patients, and an onsite chiropractic center. The Employers Health Cooperative of Wisconsin grew from one school district's desperation into 18,600 covered lives, with member groups achieving 45 percent cost reductions in year one. Ashtabula, Ohio. Anchorage, Alaska. Tuscaloosa, Alabama. The pattern is no longer rare.

**Health Rosetta didn't invent or take credit for this movement.** It is acting as a change agent, codifying what these communities did into open-source standards, supported by a certified advisor network of more than 250 and the technology infrastructure of the Nautilus Health Institute. **The proof exists. The tools exist.** The question is no longer whether a community can reclaim its healthcare river. The question is what it does with what it reclaims.

**The framework that follows shows what becomes possible in Whatcom County when even a conservative share of that extracted waste is redirected into the work that actually builds health.**

## Whatcom County Health Rosetta Dividend

### **Illustrative Investment Framework**

The framework below assumes a conservative **20 percent redirection of employer healthcare spending to local services**, on the order of \$240 million per year against the up-to-\$2 billion currently leaving the county as waste. At an average fully-burdened cost of \$80,000 to \$120,000 per role, that figure supports between 2,400 and 3,300 full-time positions. Positions in work that cannot be offshored, cannot be automated by the AI systems reshaping the economy, and address the 80 percent of health outcomes that have nothing to do with what happens inside a hospital.

Each category is tagged with the health outcome drivers it serves, using the four-factor County Health Rankings model from the University of Wisconsin Population Health Institute and the Robert Wood Johnson Foundation: *health behaviors* (~30% of outcomes), *clinical care* (~20%), *social and economic factors* (~40%), and *physical environment* (~10%). Today's healthcare dollar pours overwhelmingly into the 20 percent slice. The Dividend rebalances across all four.

| Category                                                                                                                                              | Representative roles                                                                                                                                                                                                                                                                                         | Measurable benefits                                                                                                                                                                                                                                                      | Est. roles (Whatcom)                  |
|-------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------|
| <b>Health &amp; Wellbeing</b><br><i>Preventive care, navigation &amp; recovery</i><br><i>Drives: Health behaviors, Clinical care</i>                  | Community health workers & coaches<br>Mobile preventive screening teams<br>Concierge nurse navigators<br>Crisis peer support specialists<br>Behavioral health integration coordinators<br>Recovery coaches & peer specialists<br>Veterans health navigators<br>Community paramedicine teams                  | 30-50% drop in ER visits<br>20% rise in preventive screenings<br>40% reduction in mental health hospitalizations<br>Reduced overdose deaths and veteran suicide rates<br><i>Proven: Nuka System 50% ER reduction; Ashtabula nurse navigator program</i>                  | <b>550-750</b><br><b>~\$55-75M/yr</b> |
| <b>Health &amp; Wellbeing</b><br><i>Maternal, family &amp; aging</i><br><i>Drives: Health behaviors, Clinical care, Social &amp; economic factors</i> | Doula networks (birth, postpartum, grief)<br>Perinatal mental health coordinators<br>Aging-in-place modification crews<br>Longevity program coordinators<br>Family support facilitators                                                                                                                      | Reduced maternal complications<br>25% increase in healthy life years<br>Reduced nursing home placements<br>Stronger family stability and parental engagement<br><i>Proven: Rosen Hotels high-risk pregnancy outcomes; Jonkoping County elder care model</i>              | <b>280-380</b><br><b>~\$28-38M/yr</b> |
| <b>Economic Resilience</b><br><i>Workforce, financial stability &amp; local economy</i><br><i>Drives: Social &amp; economic factors</i>               | AI transition navigators<br>Cooperative business incubator staff<br>Housing navigators & land trust managers<br>Financial literacy facilitators<br>Local food system planners<br>Trades apprenticeship coordinators<br>Veterans-to-civilian career bridge navigators<br>Manufacturing reskilling specialists | 70% worker reemployment within 6 months<br>2x growth in employee-owned businesses<br>15% increase in affordable housing units<br><i>Proven: Phifer scholarships &amp; childcare dividends; Rosen Tangelo Park economic development; Evergreen Cooperatives Cleveland</i> | <b>350-500</b><br><b>~\$35-50M/yr</b> |

| Category                                                                                                                                                                        | Representative roles                                                                                                                                                                                                                                                                                                   | Measurable benefits                                                                                                                                                                                                                                                                                                                      | Est. roles (Whatcom)                  |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------|
| <b>Skilled Trades &amp; Infrastructure Renewal</b><br><i>Builders, restorers &amp; journeyman mentors</i><br><i>Drives: Physical environment, Social &amp; economic factors</i> | Energy retrofit & weatherization crews<br>Community land trust construction crews<br>Accessory dwelling unit (ADU) builders<br>Electrical, plumbing, HVAC apprenticeship masters<br>Welding, carpentry, masonry instructors<br>Maritime trades & marine systems technicians<br>Stormwater & public works renewal crews | 30% reduction in energy-poverty-driven health claims<br>Family-wage trades careers without a four-year degree<br>Expanded affordable housing stock<br>Multigenerational skill transfer<br><i>Proven: Building Performance Association weatherization workforce; Evergreen Cooperatives; rural electric coop apprenticeship pipelines</i> | <b>350-500</b><br><b>~\$40-55M/yr</b> |
| <b>Social Fabric</b><br><i>Civic life, cultural vitality &amp; youth</i><br><i>Drives: Social &amp; economic factors</i>                                                        | Neighborhood conveners & dialogue facilitators<br>Community arts directors, muralists, theater producers<br>Restorative justice mediators<br>Youth athletic coaches & mentorship leads<br>Volunteer fire & EMS recruitment coordinators                                                                                | 35% rise in civic participation<br>20% lower crime via trust metrics<br>25% improvement in community belonging scores<br>Stronger youth engagement and adult mentorship pipelines<br><i>Proven: Rosen Hotels 80% crime reduction in Tangelo Park; Nuka community governance</i>                                                          | <b>300-400</b><br><b>~\$30-40M/yr</b> |
| <b>Environmental Renewal</b><br><i>Resilience, restoration &amp; built environment</i><br><i>Drives: Physical environment</i>                                                   | Climate resilience coordinators<br>Urban forestry & watershed restoration crews<br>Wildfire mitigation & prescribed burn crews<br>Salmon and fisheries restoration crews<br>Coastal & marine habitat restoration crews<br>Green building & energy retrofit workforce<br>Safe streets & active transportation planners  | 20% fewer health claims from environmental hazards<br>15% energy savings per household<br>25% increase in walking & cycling<br>Reduced wildfire smoke-related respiratory illness<br><i>Foundation: Jonkoping County health-environment integration; Pacific Northwest salmon recovery models</i>                                        | <b>320-440</b><br><b>~\$32-44M/yr</b> |

| Category                                                                                                                    | Representative roles                                                                                                                                                                                                                                                     | Measurable benefits                                                                                                                                                                                                | Est. roles (Whatcom)                        |
|-----------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------|
| <b>Connection &amp; Meaning</b><br><i>Learning, purpose &amp; belonging</i><br><i>Drives: Social &amp; economic factors</i> | Intergenerational tutors & mentors<br>Life transition guides (birth, grief, career)<br>Interfaith dialogue coordinators & secular chaplains<br>Local journalists & media literacy educators<br>Trades elders & journeyman mentors<br>Veterans reintegration peer leaders | 20% literacy gains; 15% elder isolation reduction<br>30% improved mental resilience post-crisis<br>25% higher voter turnout<br><i>Foundation: ReThink Health evidence on social connectedness as health driver</i> | <b>260-340</b><br><b>~\$26-34M/yr</b>       |
| <b>Total estimated community investment (conservative 20% redirection of Whatcom County healthcare spending)</b>            |                                                                                                                                                                                                                                                                          |                                                                                                                                                                                                                    | <b>2,410-3,310</b><br><b>~\$246-336M/yr</b> |

*Note: Higher redirection rates (30-50%) would support proportionally more roles, on the order of 4,000-6,000 positions at full redirection. The driver tags reference the County Health Rankings & Roadmaps four-factor model.*

## What This Looks Like in Practice

The framework above is an **implementation plan of what has already been done elsewhere**, not a wish list of programs. As the saying goes, the future is already here, it's just not evenly distributed. Every role in it already exists in a community somewhere that has reclaimed its healthcare river. Community health workers have thirty years of evidence behind them. Doula networks have measurably improved maternal outcomes. Trades apprenticeship pipelines have rebuilt family-wage careers in counties from Cleveland to rural Wisconsin. Watershed restoration crews have brought salmon back to streams that ran dry a generation ago. What is new is not the work. **What is new is the funding source.**

This plan does not dictate the services or the healthcare gaps that get filled. It provides infrastructure and inspiration. The community determines the services that matter most, the sequence of implementation, and the local partners best suited to each role. Every community will shape this differently. That is the point.

For the first time, every one of these roles can be funded without a new tax, a federal grant, or a charitable campaign. The money is already being paid. It is leaving this community every year. The choice is whether to keep accepting that, or to redirect a meaningful share of it into the human work that strengthens neighborhoods, restores skill, and addresses the actual drivers of how long people live and how well.

A natural partner for the County Health Department

Public health departments everywhere are stretched, asked to address social determinants of health, behavioral health crises, environmental hazards, and chronic disease prevention with resources that have never kept pace with the need. The Dividend brings community capacity alongside the Department, not in competition. Health departments gain partners on the ground who can extend their reach across the county. The upstream work the Department has long wanted to do at scale becomes possible. **The Department wins when this works.**

This is also how communities can respond to the wave of AI-driven workforce change now arriving. The penultimate chapter of *Relocalizing Health* frames the choice as **AI Tax versus AI Catalyst**: extraction that hits patients, workers, and employers, or redirection that empowers communities and the people who do the work AI cannot replicate. Every role in the framework above is work AI cannot replicate. Every dollar that funds those roles is a dollar redirected from the AI Tax to the AI Catalyst.

The tools exist. The proof exists. The window is open.

The Wellbeing Workforce, Mapped to What Drives Health

The roles funded by the Dividend are best understood not as a list of job titles but as a workforce mapped to the drivers of health outcomes. Below, representative role categories are organized by the four-factor County Health Rankings model. These are recognizable, fundable careers. The community will shape the exact titles and scopes that fit its needs.

| Health Driver                                                                                                                                | Representative Role Categories                                                                                                                                                                                                                                                                                                                                                                                                           |
|----------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p><b>Social &amp; Economic Factors</b></p> <p><b>~40%</b></p> <p><i>Education, income, employment, family support, community safety</i></p> | <p>Trades apprenticeship coordinators · Housing and land-trust navigators · Cooperative business incubator staff · Financial literacy facilitators · Restorative justice mediators · Family resource navigators · Intergenerational mentors · Youth athletic coaches and mentorship leads · AI transition navigators · Veterans career bridge navigators · Community arts directors · Local journalists and media literacy educators</p> |
| <p><b>Health Behaviors</b></p> <p><b>~30%</b></p> <p><i>Tobacco use, diet, physical activity, alcohol and substance use</i></p>              | <p>Community health workers and health coaches · Recovery coaches and substance use peer specialists · Behavioral health peer-support specialists · Perinatal mental health coordinators · Doulas · Community kitchen and nutrition educators · Community fitness and recreation coaches</p>                                                                                                                                             |

| Health Driver                                                                                                       | Representative Role Categories                                                                                                                                                                                                                                                                                                                                   |
|---------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Clinical Care</b><br><b>~20%</b><br><i>Access to care and quality of care</i>                                    | Concierge nurse navigators · Community paramedicine teams · Behavioral health integration specialists · Mobile preventive screening teams (dental, vision, metabolic) · Crisis peer support specialists · First responder peer-support specialists · Public health data translators                                                                              |
| <b>Physical Environment</b><br><b>~10%</b><br><i>Housing, transit, air and water quality, the built environment</i> | Weatherization and energy-retrofit crews · Affordable-housing and ADU construction crews · Electrical, plumbing, HVAC and skilled-trades apprenticeship masters · Watershed and salmon-fisheries restoration crews · Wildfire mitigation and prescribed-burn crews · Safe streets and active-transportation planners · Stormwater and public-works renewal crews |

This is a workforce that already exists in working form across the country. The organizing principle is simple: every role requires what AI cannot replicate, a human being who knows the neighborhood, earns the trust, and shows up. Communities will define the specific titles, scopes, and partnerships that fit their context.

## Sources & Evidence Base

The Health Rosetta Dividend frameworks in this companion draw on documented implementations that will be in the final version of the upcoming book, including Rosen Hotels, Ashtabula (OH) employers, the Employers Health Cooperative of Wisconsin, the Southcentral Foundation Nuka System of Care (Anchorage, AK), and thousands of others in the Health Rosetta community.

*Methodology: Whatcom County total healthcare spending estimated at \$3.9 billion (per capita figure of \$15,474 × 250,000 residents). The 15.6-cent figure is derived from BLS Occupational Employment and Wage Statistics for the Bellingham MSA cross-referenced against CMS National Health Expenditure Accounts. The 30-50% waste estimate draws on the Institute of Medicine, PwC, and three decades of Health Rosetta-aligned employer implementations.*

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### Join the Movement

Nautilus Health Institute's open-source standards, tools, and technology infrastructure are freely available at **nautilushealth.org**. Health Rosetta-certified advisors are available at **healthrosetta.org/advisors**. The full book, *Relocalizing Health: Taking Back Healthcare. Rebuilding Communities.*, launches July 29, 2026 at **RosettaFest in Nashville**. Register at **RosettaFest.org**.